HHS AI RFI Response 2026

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1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

RESPONSE:

- Regulatory ambiguity: AI utilization in care delivery is a new category - where does it fit in and what rules apply? How do we address regulatory requirements without stunting innovation?
- Safety concerns & subsequent poor adoption: Establishing safeguards to govern AI interactions and content in patient health contexts, with specific controls to prevent inappropriate diagnosis, treatment recommendations, or other clinical misuse.
- Reimbursement barriers: Reimbursement has traditionally been tied to human interactions throughout the care delivery process. Shifting certain patient interaction components to technology or AI is not formally recognized, which can limit the ability to bill & be reimbursed for services delivered through digital or hybrid models.
- Perceived risk of role displacement: The integration of technology into clinical workflows can generate apprehension among clinicians when it overlaps with their scope of care, despite persistent shortages in high-demand specialties like primary care and behavioral health.
- ROI considerations: While the technology may be cost-competitive, providers are evaluating it in the context of broader concerns and its demonstrated ability to drive improved clinical outcomes and lower total cost of care over time.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

RESPONSE:

Reduce data-sharing friction

- Clarify/standardize “safe” AI enablement under HIPAA via clearer implementation guidance on permitted uses/disclosures and de-identification pathways:
    - HIPAA permitted uses/disclosures baseline: 45 CFR § 164.502
    - De-identification standard (safe harbor / expert determination): 45 CFR § 164.514
    - Security/technical safeguards expectations (critical for AI vendors as BAs): 45 CFR § 164.312
- Modernize operational guidance for Part 2 data (SUD) to enable safe, consented exchange for whole-person AI support (where applicable): 42 CFR Part 2

Make interoperability real (so AI can work across settings)

- Strengthen enforcement + clarity around information blocking to improve access to EHI for care and evaluation (with reasonable exceptions): 45 CFR Part 171 (definition and exceptions)

3. For non-medical devices, what novel legal and implementation issues exist (e.g., liability, indemnification, privacy, security) and what role, if any, should HHS play to help address them?

RESPONSE:

Liability ambiguity: “Who’s responsible” when AI advice contributes to harm - developer vs. clinician vs. health system (especially with continuously learning systems). The need to clearly define where liability of one participant in a care journey ends and the next begins will inform regulatory needs & requirements for each participant.

Indemnification + contracting: Health systems increasingly demand strong indemnities, audit rights, model-change notices, and incident response SLAs.

Privacy/security: Training, logging, and telemetry can expand PHI exposure; needs BAAs, least-privilege access, and clear retention policies.

IP + data rights: Who owns derived features, fine-tuned weights, and performance telemetry?

HHS role: Provide model governance playbooks, standard contract clauses, and best-practice guidance for PHI-minimizing architectures (without forcing everything into “device” pathways).

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and workflow/human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, prize competitions)?

RESPONSE:

For artificial intelligence systems used in clinical care, particularly those intended to support or inform clinical decision making, rigorous pre and post deployment evaluation is essential regardless of regulatory classification. Evaluation should begin with clear articulation of intended use, target population, clinical setting, and the specific decisions the system is designed to influence. It is also important to clearly define the level of autonomy the system is intended to exercise, ranging from informational support to conditional recommendation to more independent action within defined boundaries.

Pre deployment assessment should include retrospective validation across diverse and representative datasets, external testing in independent health systems, and careful analysis of performance across clinically relevant subgroups. In addition to measures of discrimination, evaluation should assess calibration, clinically meaningful operating thresholds, and decision analytic impact. Prospective evaluation methods such as silent deployments, pragmatic trials, or cluster based study designs can provide insight into real world performance before widespread rollout.

Safety must be addressed through both technical safeguards and operational controls. From a technical perspective, systems should undergo structured hazard analysis to identify foreseeable failure modes, including incorrect recommendations, omissions, automation bias, hallucinated outputs, and performance degradation under distribution shift or missing data. Where systems are designed to act with greater independence, guardrails should include clear scope limitations, rule based constraints aligned with clinical guidelines, confidence thresholds that trigger human review, and automatic fallbacks when inputs fall outside validated conditions. From an operational perspective, organizations should establish real time monitoring and escalation pathways

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, credentialing) to promote innovative and effective AI use in clinical care?

RESPONSE:

Establish voluntary, lightweight certification focused on: safety case, monitoring plan, transparency, and interoperability readiness (not “one-size-fits-all”).

Fund/endorse independent test labs and publish minimum reporting standards (like “model cards” + clinical workflow cards).

Encourage industry-led credentialing for “AI safety officer / clinical AI steward” roles inside provider orgs.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

RESPONSE:

Met/exceeded

- Imaging/screening triage + radiology support (strong ROI when it reduces second reads or speeds throughput). Recent evidence includes AI-supported mammography screening trials showing noninferiority/safety in population screening.
- Documentation burden reduction (“ambient scribe” category) showing measurable time savings and clinician experience improvements in real-world/QI and trials.

Fallen short

- Predictive Analysis
  - Sepsis prediction/early warning in some settings due to poor external validity, alert fatigue, and workflow mismatch (Epic model has published external validations showing limitations).

Highest-potential novel tools

- **Digital Behavioral Health Tools:** Use AI to deliver safe, protocol-based care (e.g., Cognitive Behavioral Therapy) at scale, expanding access and integrating behavioral health into chronic disease management to address the mental health gaps that worsen outcomes.
- **Automated Referral Coordination:** Streamline referrals by automating intake, data routing, scheduling, and returning consult notes to the ordering provider — including connections to community resources. The biggest barrier is fragmented EMR integration across systems; solving this would enable seamless, end-to-end care.
- **Safety Analyzers:** Use AI to review clinic conversations and notes to detect hidden medical risks before they lead to ED visits.

7. Which roles, decision makers, or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to adoption?

RESPONSE:

Influencers

- CMIO/CNIO/CTO, clinical department chairs, quality/value-based care leadership,

Primary hurdles

- Security review, EHR integration (SMART/FHIR + workflow), clinical governance approval, safety protocols, and proving ROI under real constraints (staffing, bandwidth).

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

RESPONSE:

Data types: longitudinal meds, labs, problem lists, imaging metadata, referrals, scheduling, utilization/claims, SDOH, and patient-reported outcomes.

Standards: Expand consistent use of **FHIR** resources + USCIS-aligned data quality, plus event-driven APIs for near-real-time updates. Provide transition tools and methodologies for systems with more legacy EHR implementations to FHIR standards. Explore usage of Model Context Protocol (MCP) as an optional access method. Continue to incorporate human-in-the-loop safety monitoring and escalation frameworks.

Benchmarking: shared, de-identified evaluation corpora for (a) longitudinal prediction, (b) summarization safety, (c) care-gap detection, with subgroup reporting and drift checks.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? What concerns do patients and caregivers have related to adoption and use?

RESPONSE:

Wants addressed

- Faster access, fewer delays, clearer care plans, reduced paperwork, and proactive outreach (especially for chronic care).

Concerns

- Patient safety, Data privacy, bias/inequity, over-automation (loss of human judgment), job displacement, and unclear recourse when AI is wrong.

10. Are there specific areas of AI research that HHS should prioritize to accelerate adoption of AI as part of clinical care?

a. Are there published findings about the impact of adopted AI tools and their use in clinical care?

b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

RESPONSE:

Priority research areas

- Pragmatic, multi-site clinical trials of AI-in-workflow (not just retrospective AUC).
- Bias + equity measurement standards tied to real outcomes.
- Efficacy of AI- vs human-delivered mental health care (cognitive behavioral therapy) and the downstream impacts of integrated care via AI on clinical goals of chronic condition management

a. Published findings on impact

- Dr. Jessica Schleider has completed studies comparing virtually-delivered to human-delivered single session interventions, showing that outcomes are similar in both groups. Research study is currently being designed to extend this hypothesis exploring different treatment delivery modalities to compare AI-delivered care to human delivered care specifically for single session.
- [other AI in care delivery studies]

b. Costs/benefits/transfers in the literature

- Benefits often show up first as time saved, throughput, and clinician experience, while financial ROI can lag or shift across stakeholders (provider vs payer).